



OFFICIAL

Management of Patients with Eating Disorder Procedure

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1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	Medical, Mental Health and Community Services	All disciplines

2. Purpose

To outline a clear pathway for the management of inpatients with an eating disorder across the Mental Health Unit (MHU) and medical wards at Armada Health Service (AHS) through to discharge to community services.

3. Background

Emerging evidence suggests early intervention, including assertive nutritional rehabilitation and structured psychological support with a multidisciplinary plan for relapse prevention, are crucial for improving prognosis for patients with eating disorder.

Severe malnutrition and starvation syndrome can be experienced in people at any weight, resulting in physical and cognitive deficits including poor insight regarding nutritional and medical risk. The use of the WA Mental Health Act 2014 may be required to facilitate safe treatment. Severely malnourished individuals are unlikely to be able to reverse starvation on their own.

Patients with high-risk medical complications or recurrent presentations may require inpatient admission to enable nutritional rehabilitation and weight gain to a level sufficient for brain recovery from the atrophic changes of malnutrition. Otherwise, this may continue to drive eating disorder cognitions and impair engagement with psychological treatments, thereby perpetuating the cycle of recurrent weight loss and readmission.

Furthermore, starvation syndrome is associated with adverse mental health consequences which include, but are not limited to, anxiety, low mood, and suicidal ideation.

4. Goals of admission

Medical and Mental Health Consultation Liaison Service (MHCLS) Psychiatry to agree on the goals of admission from the outset, focussing on the patient's overall presenting state considering:

- Medical stabilisation
- Prevention or treatment of refeeding syndrome
- Nutritional rehabilitation with weight restoration as appropriate
- Reversal of cognitive effects of starvation so the person can engage in psychotherapy
- Consumption of adequate nutrition and containment of compensatory behaviours both on and off the ward prior to discharge
- Appropriate outpatient follow-up care

5. Indicators for Admission

5.1 Admission Criteria

To determine if the patient requires an inpatient medical or psychiatric admission, refer to [Table 1: Admission Criteria](#) outlined below.

If a psychiatric admission is indicated, in consultation with the Mental Health Emergency Department Liaison Service (MHEDLS) determine if the patient is suitable for:

- AHS Mental Health Unit (MHU) admission *OR*
- Referral to Cockburn Health Eating Disorder Unit. Refer to [Appendix 1](#) for the inclusion criteria and referral process.

5.2 Admission Criteria Not Met - Medical or Psychiatric

If the patient does not meet criteria for an inpatient admission, refer to [Appendix 2](#) for community eating disorder services. WA Eating Disorder Outreach Consultation Service (WAEDOCS) is available to discuss appropriate community pathways on 1300 620 208 (9am to 4pm Monday to Friday).

5.3 Admission from AHS Mental Health Community Services

If the patient is known to AHS Mental Health Community Services and becomes medically unstable (refer to the criteria outlined below in [Table 1: Admission Criteria](#)) the Community team can make a referral for direct admission to the MAU by contacting the on-call MAU Consultant and placing the referral on the Enterprise Bed Manage System (EBM).

The on-call MAU Consultant will:

- Provide medical advice to guide the physical management of the patient.
- Consider the acuity of the patient and decide if the patient will be admitted directly to the MAU or if they should attend the Emergency Department.
- Discuss the referral with the MAU team.
- Organise for the outcome of the referral to be communicated to the referrer in a timely manner.

Table 1: Admission Criteria (Adapted from WAEDOCs and QuEDS Guidelines)

Medical Parameters		Medical Admission	Psychiatric Admission
Physical observations	Blood pressure	<80mmHg systolic OR - postural blood pressure >20 mmHg drop (lying to standing) <17 years <70mmHg Systolic BP OR postural blood pressure >10 mmHg drop (lying to standing)	<90mmHg systolic OR - postural blood pressure >10 mmHg drop (lying to standing)
	Heart rate	≤40bpm (adolescents <50 bpm) or >120bpm OR - postural tachycardia >20 bpm (i.e. increase in heart rate >20 bpm from lying to standing) <17 years <50bpm or increase of over 10bpm on standing	
	Temperature	<35.5°C OR cold/blue extremities	
	ECG	Any arrhythmia including QTc prolongation, non-specific ST or T-wave changes, including inversion or biphasic waves	
	Blood sugar	BGL <3.5 mmol/l	
Pathology	Sodium	<120mmol/L	<130mmol/L
	Potassium	Below 3.5 mmol/L	
	Magnesium	Below 0.7 mmol/L	
	Phosphate	Below 0.75 mmol/L	
	Albumin	<30g/L	
	LFT	Mildly elevated (AST or ALT >500)	
	Neutrophils	<0.7 x 10⁹/L	<1.0 x 10⁹/L
	eGFR	<60ml/min/1.73m² or rapidly dropping (25% drop within a week)- Alb: <30g/	
Anthropometry	Rate of weight loss	Loss of > 1kg/week over several weeks	
	BMI	- BMI < 14kg/m² (for >18 years) - Below <75% Ideal Body Weight (<18 years)	14 – 16 kg/m²
Nutritional	Oral intake	Grossly inadequate nutrition intake (<1000kcal/day) for > 2 days	
Compensatory behaviors	Vomiting, laxatives, exercise	Unmanageable compensatory behaviours (vomiting >4x/day, excessive daily laxative use)	
Other	Severity of symptoms	Not responding to outpatient treatment	

6. Treatment in the Emergency Department

Table 2: Treatment Guide in the Emergency Department While Awaiting Transfer to a Medical Ward or MHU			
	Orders	Awaiting Medical Admission	Awaiting Mental Health Admission
Pathology	FBC, LFTs, Phosphate, Mg, eGFR	Daily	Second daily
	TFTs B12, folate, Ca+2	Initial	As clinically indicated
	ECG	Daily	Second daily
Supplementation <i>Refer to RPBG Refeeding Syndrome Clinical Guideline</i>	Thiamine	300mg daily IM/IV prior to feeding and daily for the first 3 days	-
	Multivitamin	Daily	-
	Electrolytes (K+, PO4, Mg)	Replace if abnormal before feeding	-
Observations	BGLs (capillary)	QID 1-2 hours <u>post</u> meals AND 02:00	BD (<i>Can cease if continue to be within normal range</i>)
	Postural BP + HR	QID Ensure 2 minutes apart (lying to standing)	BD
	Temperature	QID	BD
	Fluid balance chart	Including IV, NG, oral food/fluid. Include output if medically indicated	Including IV, NG, oral food/fluid. Include output if medically indicated
	Weight	Daily	Twice weekly (BLIND)
	Height	Record initially (do not accept reported)	Record initially (do not accept reported)
	Bowel chart	Daily	Daily
	Food intake chart	Observe and document at each meal and snack	Observe and document at each meal and snack
	Compensatory behaviours	Vomiting, laxative use and exercise/movement	Vomiting, laxative use and exercise/movement
Nutrition	Early commencement of nutrition should be managed by the Emergency Department team.	Continuous enteral feeding is to commence while in the Emergency Department if <u>ANY</u> of the medical admission criteria are met. (refer to Table 1: Admission Criteria) Refer to Appendix 3 for the Nutritional Management Pathway	While awaiting a psychiatric bed, it is important that a nutrition plan is commenced to maintain medical stability. Refer to Appendix 4 Meal Plan A (8000 kJ)

7. Management on the Medical Ward

The medical team are responsible for referring to MHCLS via e-referral and Dietetics via page 148 (MAU) or 225 (Canning) as soon as the patient is admitted to the medical ward. Refer below to [Table 3: Management on the Medical Ward](#) for medical and nursing interventions.

Table 3: Management on the Medical Ward			
	Orders	Frequency at commencement	Reduction as clinically indicated after initial refeeding period
Pathology	FBC, LFTs, Phosphate, Mg, eGFR	Daily for at least the first 7-10 days	Second daily until week 3 and then twice weekly on weigh days
	TFTs B12, folate, Ca+2	Initial (may not be required if ordered in ED)	As clinically indicated
	ECG	Daily	Second daily or twice weekly
Supplementation	Thiamine	300mg daily IV first 3 days	100mg oral daily (after day 3)
	Multivitamin	Daily	Daily
	Electrolytes (potassium, Phosphate, Magnesium)	Replace if abnormal before feeding	When levels are maintained
	Refer to RPBG Refeeding Syndrome Clinical Guideline available on the hub		
Observations	BGLs (capillary)	QID 1-2 hours <u>post</u> meals AND 02:00	BD Can cease if continue to be within normal range
	Postural BP + HR	QID Ensure 2 minutes apart (lying to standing)	BD
	Temperature	QID	BD
	Fluid balance chart	Including IV, NG, oral food/fluid. Include output if medically indicated	
	Weight (post-void, before breakfast aiming for 06:30)	Daily (BLIND)	Twice weekly (BLIND)
		Be mindful that the patient is always listening, and it should be decided as a team how often weights are discussed to support progress	
	Bowel chart	Daily Avoid use of senna-based stimulant laxatives Nursing to enquire and monitor using stool chart	
	Food intake chart	ALL food and fluid to be observed and document at each meal and snack	
	Meal Plan	The Shift Nurse will oversee the meal plan and discuss with the Shift Coordinator if support is required	
	Compensatory behaviours	Monitor purging, concealment of food, and unauthorised movement	
Other	IV Dextrose should not be necessary. If it becomes necessary, it must be accompanied by IV Phosphate and Thiamine simultaneously		

8. Management in the Mental Health Unit (MHU)

Refer below to [Table 4: Management in the MHU](#) for medical and nursing interventions while in the MHU.

Table 4: Management in the MHU		
	Orders	Frequency
Pathology	FBC, LFTs, Phosphate, Mg, eGFR	Second daily until week 3 and then twice weekly on weigh days
	TFTs B12, folate, Ca+2	As clinically indicated
	ECG	Second daily or twice weekly
Supplementation	Thiamine	100mg oral daily (after day 3)
	Multivitamin	Daily
	Electrolytes (potassium, Phosphate, Magnesium)	When levels are maintained
		Refer to RPBG Refeeding Syndrome Clinical Guideline available on the hub
Observations	BGLs (capillary)	BD (<i>Can cease if continue to be within normal range</i>)
	Postural BP + HR	BD
	Temperature	BD
	Fluid balance chart	Including IV, NG, oral food/fluid. Include output if medically indicated
	Weight (<i>post-void, before breakfast</i>)	Twice weekly (BLIND)
		Be mindful that the patient is always listening, and it should be decided as a team how often weights are discussed to support progress
	Bowel chart	Daily <i>Avoid use of senna-based stimulant laxatives Nursing to enquire and monitor using stool chart</i>
	Food intake chart	ALL food and fluid to be observed and document at each meal and snack
	Meal Plan	The Shift Nurse will oversee the meal plan and discuss with the Shift Coordinator if support is required
	Compensatory behaviours	Monitor purging, concealment of food, and unauthorised movement
MEAL PLAN: On arrival to the MHU, the Shift Coordinator will liaise with the kitchen to ensure the meal plan is continued. The patient is expected to comply with 100% of the meal plan. If there is refusal of step 2, the MHCL team will request insertion of an NGT within 24 hours so that step 3 can be implemented. Refer to Appendix 3 Nutritional Management Pathway for Eating Disorders if this occurs after hours/weekends. ENTERAL NUTRITION: If a patient is transferred with an NGT or requires insertion of an NGT while on the MHU, Nursing from the Medical team will provide outreach support for insertion and management. It is the responsibility of the MHU NUM to liaise with the Medical NUM to organise.		

9. Medical deterioration/ escalation

Nursing should inform the RMO immediately if one or more parameters are met:

- Pulse is below 50bpm
- Systolic BP below 90mmHg (unless modifications are in place on AKMR140.3)
- Significant Postural Drop of more than 10mmHg
- Postural Tachycardia >20bpm
- QTC interval >450ms
- BGLs <4.0mmol/L
- Temp <35.5

10. Nutritional Management

Nutritional rehabilitation is managed by Dietetics and is based on the therapeutic BMI set by the MHCLS.

10.1 Continuous Enteral Nutrition

For medically unstable patients, [Appendix 3: Nutritional Management Pathway for Eating Disorders](#) should be commenced in the Emergency Department and continued by Dietetics when transferred to the medical ward.

When the patient is admitted to the ward, the Dietitian will complete a nutritional assessment and prepare the *AKMR 60.3 Enteral Feeding Regime* form.

Continuous feeding is recommended to reach the energy goal and may be required for a further 48 hours if the patient remains medically unstable.

10.2 Transitional Regime

Clinical reasoning can be used to determine when transitioning from continuous feeds is appropriate, and the information provided should be used as a guide only.

To transition from continuous feeds consider if the patient is:

- Medically stable:
 - Commence 3-step meal plan
 - NGT to remain in place until it is not used for 3 consecutive days
 - The team may recommend keeping the NGT in place for longer
- Not yet medically stable:
 - Transition to overnight feeding in conjunction with the 3-step meal plan to meet the overall energy goal *or*
 - Determine if continuous feeding should continue

10.3 Standardised Meal Plans

Standardised meal plans (8 MJ – 16 MJ) will be implemented by the Dietitian to support effective and efficient nutritional rehabilitation. The meal plans have been developed so that the patient has a choice while supporting therapeutic boundaries.

The meal plan has 3 steps allowing the patient to:

- Consume food orally (step 1) *or*

- Drink an oral nutritional supplement (step 2) *or*
- Have an enteral feed top up (step 3)

The Dietitian will:

- Liaise with the patient before the meal plan is increased towards the energy goal.
- Avoid modifying the standardised meal plan and ensure any approved swaps are equal in energy value as per the department ready reckoner.
- Call the Catering Department to request a specific meal plan and document on iCM.
- Provide a copy of the meal plan to the patient and place a copy in the patient's bedside file. If any modifications are made to the meal plan, a copy will also be provided to the Catering Department.

The FSA (Food Service Attendant) completing the daily menu slip will use the standardised meal plan to support main meal selections and will not deviate from what is listed. The supporting nurse on shift (or allocated AIN) is responsible for liaising with the FSA to ensure the snacks listed on the meal plan are provided.

The patient will not be able to request changes outside of the scheduled Dietetic review and should be encouraged to write down requests for discussion.

10.4 Discharge Meal Plans

The Dietitian will use the WAEDOCS standardised community meal plans and provide education to the patient and/or their support person prior to discharge.

11. Mealtime Support

- It is important to create a supportive mealtime environment.
- Ask the patient if they have any strategies which help reduce heightened emotions during mealtimes.
- If the patient is not allocated a 1:1 special, it is still important for the patient to receive 1:1 mealtime supervision from an AIN or RN during meal and snack times.
- Toilet breaks are not permitted during meals.
- Observe for purging/chewing/spitting or attempting to conceal/discard food/fluid.
 - If observed, then it is important to immediately address this with the patient in a non-judgemental manner and progress to step 2 of the meal plan if indicated.
- Do not deviate from the meal plan.
 - 100% of the food listed on the meal plan is expected to be consumed.
 - The patient is always made aware of this expectation, and it is important Nursing staff uphold this therapeutic boundary in a supportive manner.
- Be aware of the time allocated for meals/snacks documented on the meal plan. After this time if the patient has not consumed 100% then progress to the next step of the meal plan.
- Refer to [Appendix 5](#) for Therapeutic Communication.

12. Transfer Criteria Within Armadale Hospital

12.1 Criteria for transfer to AHS MHU

The patient should be medically stable for a minimum of 48 hours prior to transfer to the MHU:

- Systolic BP 90mm (OR in the case of systolic BP between 80mmHg and 90mmHG if agreed in conjunction with Psychiatrist).
- Heart Rate >50 and <100 bpm.
- No significant postural tachycardia or hypotension.
- Normal ECG.
- Normal electrolytes.
- Stable BGLs that are within the normal range.
- BMI to increase by ≥ 2 bands and ideally >14 kg/m².
- Reached goal nutrition target for at least 48 hours (and is at least 7 days from commencement of refeeding).

The Medical Consultant should discuss all mental health patients and comorbid eating disorders, with the treating Psychiatry Consultant before transfer decisions are made.

Out of hours transfers should occur rarely and only in discussion between on-call Consultant Psychiatrist and on-call Medical Consultant.

Repatriation to a mental health bed will be prioritised if further specialist psychiatric management is required.

12.2 Criteria for Transfer to a Medical Ward

If the patient becomes medically unstable (refer to [Table 1: Admission Criteria](#)) while on the MHU, the Consultant Psychiatrist must refer to the MAU or Medical Consultant on call. The outcome of the referral should be decided and communicated to the referrer in a timely manner. If a patient requires a length of stay greater than 48 hours, then patient care will be transferred to an ongoing medical bed for further management.

13. Discharge from Armadale Hospital Medical or MHU

When the patient is deemed medically stable (refer to section 13.2 *Criteria for Discharge to the Community*), there will be collaboration between the MHCL team and Dietetics to determine the appropriate community service for eating disorder treatment and management. It is the responsibility of the MHCL Psychiatry team to ensure a referral is made to the appropriate eating disorder service prior to discharge.

13.1 Step-down to Cockburn Health Inpatient Eating Disorders Unit

If the patient is medically stable but still requiring nutritional rehabilitation, they may be suitable for step-down to Cockburn Health Inpatient Eating Disorders Unit. Refer to [Appendix 1](#) for the inclusion criteria and referral process.

13.2 Criteria for Discharge to the Community

Discharge to the community can be considered if the patient is medically stable **PLUS**:

- Demonstrated regular adequate nutritional intake on and off the ward, while maintaining weight.
- If on MHU, returns from 48 hours leave medically stable.
- Cognitive effects of starvation have reduced sufficiently for the patient to be able to engage in outpatient psychotherapy (will typically require a BMI >16 kg/m²).
- MHCL Psychiatry have been involved and approve of discharge to the community.
- Dietetics have provided a discharge meal plan.

13.3 Public and Private Community Eating Disorder Services

Refer to [Appendix 2](#) for public and private options available in the Perth Metro area.

14. Roles and Responsibilities

Role	Responsibility
Mental Health Consultation Liaison Service (MHCLS)	MHCLS will see patients referred on the medical wards and will remain involved in psychiatric management throughout the admission, which includes: • Psychiatric assessment, treatment, and management plan. • Management of Mental Health Act (2014) Forms. • Behaviour management plans. • Clinical psychology involvement as required. • Liaison with relevant mental health services prior to transfer or discharge. The MHCLS plays an integral role in discharge planning for patients with eating disorders. This involves liaising with the patient and the MDT and advising on transfer to mental health units or identifying suitable community-based services once medically stable. Additionally, the MHCLS Psychiatry team provides regular and frequent support (up to daily if required) to assist with behavioural and psychological management. This may include patient and family interviews, guidance to medical staff on managing challenging or compensatory behaviours, and support for mental health stabilisation. Refer to Appendix 6 for a guide to applying the Mental Health Act 2014
Nursing	• Complete observations and record in the Comprehensive Care Plan. • Administer enteral nutrition. • Implement the meal plan.

Role	Responsibility
	- Escalate clinical deterioration to the medical team (refer to section 9 Medical Deterioration/ escalation). - Oversee 1:1 support especially at meal times. - Engage in therapeutic communication. - Liaise with Nursing Coordinator regarding any challenges with patient load/burn out. - Nursing Coordinator to consider room location to allow for adequate observation and support.
Dietitian	- Complete an initial assessment including eating disorder history, current and past compensatory behaviours, diet history, weight history, assess malnutrition status, determine risk of refeeding syndrome and calculate nutrition requirements. - Determine the rate of nutrition rehabilitation and consider if this requires continuous enteral nutrition or 3 step meal plan. - Facilitate the transition from enteral nutrition to 3 step meal plan to meet energy requirements. - Monitor weight restoration and guide BMI targets. - Reviews will be scheduled on alternating days unless a change to the nutrition intervention is required more frequently (this will be determined by the Senior Dietitian). - Collaborate with MHCLS regarding readiness for step-down/discharge and appropriate community support. - Provide a discharge meal plan.
Medical Team	The medical team will treat medical complications and restore medical stability in the early stages of nutrition rehabilitation, this includes overseeing the management of refeeding syndrome. They will provide medical outreach support for the psychiatry team when the patient is admitted to the MHU.

15. Policy context

The following documents are required to give effect to this procedure:

- Legislative
 - WA Mental Health Act 2014
- HDWA
 - [Recognising and Responding to Acute Deterioration Policy](#)
 - [Clinical Handover Policy](#)
 - [State-wide Standardised Clinical Documentation SSCD for Mental Health Service](#)
- EMHS
 - [Acute Response Mental Health Emergencies/Crisis \(EMHS: 17\)](#)
 - [Discharge Against Medical Advice \(EMHS: 4\)](#)

16. Supporting information

The following documents support the implementation of this procedure:

- [Mental Health Consultation-Liaison Service](#)
- [Recognition and Response to Acute Deterioration](#)
- [Consumer Safety Planning Procedure](#)
- [Observations – Physiological](#)
- [Safe and Supportive Observations Procedure](#)

17. Compliance, monitoring and evaluation

Compliance against this procedure will be evaluated by the Mental Health Safety and Quality Departmental meeting and document will be maintained by Coordinator Dietetics.

18. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards ([second edition](#)):

- Standard 5 – Comprehensive Care - Action 5.3, 5.5, 5.7, 5.10 and 5.28

[National Standards for Mental Health Services](#)

- Standard 9 - Criteria 9.2
- Standard 10 - Criteria 10.2.1
- Standard 10 - Criteria 10.3.3
- Standard 10 - Criteria 10.5.2
- Standard 10 - Criteria 10.5.11
- [Chief Psychiatrist's Standards for Clinical Care](#)

19. Bibliography

1. North Metropolitan Health Service (NMHS) [Eating Disorders: Assessment, Admission and Initial Management – A Quick Reference Guide](#)
2. Government of Western Australia [Centre for Clinical Interventions \(CCI\)](#)
3. Royal Australian and New Zealand College (RANZC) of [Psychiatrists Clinical Practice Guidelines for the Treatment of Eating Disorders](#)
4. MARSIPAN: [Management of Really Sick Patients with Anorexia Nervosa](#)
5. [Royal Perth Bentley Group \(RPBG\) Refeeding Syndrome Clinical Guideline](#)
6. [Royal Perth Bentley Group Eating Disorders: Management of Patients SOP](#)
7. [QuEDS Guide to Admission and Inpatient Treatment | Metro North Mental Health | Metro North HHS](#)

20. Glossary

The following definitions are relevant to this policy.

Term	Definition
FSA	Food Service Attendant
MAU	Medical Admission Unit
MHCLS	Mental Health Consultation Liaison Service
MHEDLS	Mental Health Emergency Department Liaison Service
MHU	Mental Health Unit
WAEDOCS	WA Eating Disorder Outreach Consultation Service

21. Policy owner (relating to these procedures)

Executive Director – Armada Kalamunda Group

Enquiries relating to this procedure may be directed to:

Title: Coordinator Dietetic & Nutrition
Department: Dietetics & Nutrition
Email: Linda.Freeman@health.wa.gov.au

22. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	30 03 2026	30 03 2029	Full review, significant changes throughout document.

23. Authorisation

This procedure has been authorised and issued by:

Authorised by	Alisha Thompson – Executive Director (AKG)
Authorisation date	26 03 2026
Published date	30 03 2026

24. Appendices

24.1 Appendix 1: Cockburn Health Eating Disorder Unit

24.1.1 Overview

Cockburn Health Eating Disorders Service provides bi-directional transition between services for patients that can't be managed in the community or are inpatients at other facilities that were admitted for medical / mental health instability and are currently stable but would benefit from a higher level of treatment and support before stepping down to community services. The focus of care being on psychological interventions, nutritional rehabilitation and normalisation of eating.

24.1.2 Referral process and inclusion criteria

Referrals can be made via EBM for **medically stable** patients (**women only**) who require nutritional rehabilitation. Referrals can be made from the Emergency Department or inpatient medical/psychiatry wards if the patient is:

- Experiencing a primary diagnosis of an eating disorder with or without mental health co-occurring.
- Agreeable to following a “meal plan” or “able to eat” or demonstrated ability to meet nutritional needs orally, with or without NGT feeding.
- Voluntary according to Mental Health Act Legal Status.
- Low risk of suicide.
- Low risk of aggression.
- Under 18 years of age they will be considered on a case-by-case basis.

24.2 Appendix 2: Community Eating Disorder Services

24.2.1 East Metro Eating Disorder Specialist Service (EMEDSS)

The programs available include:

- **Intensive Clinical Monitoring:** nurse-led service provides nutritional and physical health monitoring and motivational support over eight weeks with up to four appointments per week
- **Specialist Multidisciplinary Outpatient Clinic:** Staff in the clinic can offer therapies, transition support, relapse prevention planning and support for general health and wellbeing over a 12-week period, as determined by clinical need.

Inclusion criteria:

- Agreeable to engaging with the service and willing to attend weekly appointments as a minimum.
- Aged 16 years and older.
- Eating disorder is a diagnosis of primary concern.
- Resides within the EMHS catchment, including WACHS catchment.
- Medically stable - can be maintained in the community or out of hospital.
- Engaged with a GP in the community or is willing to do so.

Referrals can be made by a Psychiatrist/Psychiatry Registrar [EMEDSS--Referral-Form-PDF--March-2025.pdf](#)

24.2.2 Butterfly Foundation

Butterfly *Next Steps* is a free, online, intensive outpatient program, offering support after an inpatient admission for an eating disorder. The program runs for 3 hours per day, 3 days per week for 8 weeks by a trained team:

- One-on-one sessions with a psychologist, a dietitian, and a peer worker each week.
- Support with meals.
- Group sessions with other participants.
- Weekly learning and support groups for families and carers.
- Regular meetings with a nurse and GP or psychiatrist to make sure the treatment plan stays consistent.

Inclusion criteria:

- Over the age of 16 years.
- Meet DSM-5 diagnostic criteria for AN, BN, OSFED, BED as a primary diagnosis.
- Be discharging from a residential or inpatient unit in line with medical advice, or have had a planned discharge from residential/inpatient unit in the 90 days prior to referral to *Next Steps*.
- Be medically stable at intake, assessment, and for the duration of the program.
- Be willing to undergo regular medical monitoring for the duration of the program.
- Have well managed co-occurring substance-use disorders.
- Maintain weight restoration or maintenance in line with the discharge plan and recommendations from the inpatient or residential unit.

- Be assessed a low risk for suicidality or deliberate self-harm.
- Be able to participate in the program virtually.
- Have access to a private and confidential space where can participate in the sessions.

Prior to discharge, the treating Psychiatrist can complete the referral form ([Next Steps Outpatient Program - Butterfly Foundation](#)). The patient must read the participant handbook ([Next-Steps-Participant-Handbook-2025.pdf](#)) and sign the treatment consent forms ([Microsoft Word - Next Steps Consent Form](#)) prior to the referral being sent.

24.2.3 Private Eating Disorder Services

- **Hollywood Private Hospital** [Eating Disorder Treatment Perth | Ramsay Clinic Hollywood](#)
 - ED inpatient program
 - ED day program
- **Swan Centre** [Our Services - The Swan Centre](#)
 - Individual ED treatment
- **Centre for Clinical Interventions** [CCI Eating Disorders Treatment - Perth](#)
 - Group Program
 - Individual ED treatment
 - Family Based Therapy for Anorexia Nervosa
 - Eating Disorder Support + Skill Building Group for Family and Friends
- **ESUS Centre** [Intensive treatment program \(ITP\)](#)
 - Intensive treatment Program

Medicare provide an Eating Disorder Care Plan which allows 40 appointments with Psychology and 20 appointments with Dietetics, with a GP overseeing medical stability. This allows the patient to select their own treating team.

24.2.4 AHS Community Mental Health Service

Patients who are referred to AHS community mental health services with eating disorders are assessed and treated on a case-by-case basis. They do not provide a specialised eating disorder program but can provide additional support for clients with co-occurring complex mental health needs who are also attending EMEDSS or an alternative service.

24.3 Appendix 3: Nutritional Management Pathway for Eating Disorders

The aim of early commencement of nutrition is to allow for timely progression to medical stability. The Nutritional Management Pathway is intended to be implemented by the medical team in the Emergency Department and continued by Dietetics.

1. Medical team to determine suitability for this pathway based on parameters outlined in **Table 1: Admission Criteria**
2. Medical team to liaise with Nursing team to organise the placement of a nasogastric tube (use a fine bore 12Fr tube). Refer to *AKG Enteral Feeding Enteral Tubes Management – Adult Procedure*.
<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/EMHS/AHS/CPS.NPM.Enteral.pdf>
3. Medical team to ensure prophylactic supplementation is commenced and deranged electrolytes are replaced prior to the commencement of enteral nutrition (refer above to **Table 2: Treatment in the Emergency Department**)
4. Medical team to print this page, write the date/time, and provide to Nursing staff to follow.
5. Follow the feeding regime as outlined below (despite refeeding risk).
6. The medical team (or Dietitian) should determine if there are any contraindications to increasing the feed rate on day 3 and 5. The medical team to check and replace electrolytes before increasing the feeding rate outlined below.

Nutritional Management Pathway for Eating Disorders				
<i>Enteral feeding should only commence after tube placement is confirmed, supplementation is charted, and electrolyte replacement has commenced (if indicated)</i>				
Enteral Product	Isosource 1.5 (250 ml tetra)			
	Day 1	Day 3	Day 5	Day 7 onwards
	Date: Time:	Date: Time:	Date: Time:	Date: Time:
Feed Rate	40 ml/hour	55 ml/hour	70 ml/hour	80ml/hour
Water Flush	50ml every 4 hours			
Feeding Period	Continuous over 24 hours with Kangaroo pump			
Total Feed volume	960 ml	1320 ml	1680 ml	1920ml
Total Flush volume	300 ml	300 ml	300 ml	300ml
Total fluid	1260 ml	1620 ml	1980 ml	2220 ml
Nutrition	6086 kJ	8368 kJ	10,651 kJ	12,170kJ
• Oral Intake: NBM, limit fluid intake to 250ml water/day • Hydration/flushes: May require IV fluids or alteration to fluid flushes to meet fluid requirements (40ml/kg/day) • Confirmed food allergy (not including soy) use Fortisip Plant Based (960ml=6048kJ, 1320ml=8316kJ, 1680ml=10,584kJ, 1920ml=12,096kJ) • Special dietary considerations (renal impairment or intestinal failure) discuss with Dietetics • The coordinator can organise a PCA to collect a carton of Isosource 1.5 or Fortisip Plant Based from RAC • Dietitian Referral: Page 148 or 225 during business hours and include your name and extension number. After hours call 2536 and leave a message. The referral will be prioritised on the next working day				

24.4 Appendix 4: Meal Plan A

Early commencement of nutritional rehabilitation ensures medical stability is maintained. Meal Plan A provides ~8000 kJ and is intended to be implemented in the Emergency Department while the patient awaits transfer to the MHU.

Reported food allergies should be assessed by the Registrar to determine if a food allergen has been formally diagnosed. A formal diagnosis must be adhered to, and clinical judgement should be used for other dietary restrictions.

The Catering Department can provide the standard hospital menu or a vegetarian menu (contains dairy) and will accommodate confirmed food allergies. No other special requests or food preferences can be catered for while in the Emergency Department.

1. Medical team to confirm the patient is medically stable and meets criteria for a psychiatric admission (Refer to **Table 1: Admission Criteria**).
2. Nursing will make a once-off call to the Catering Department to request Meal Plan A
 - o Meal ordering cut off times **Breakfast:** 6:15 am, **Lunch:** 10:15 am, **Dinner:** 3:15 pm
3. Nursing will organise a PCA to collect a carton of Fortisip or Fresubin Energy from RAC. If a food allergy is confirmed please liaise with Dietetics for an alternative option.
4. Nursing will commence a food intake chart (form AKMR 60.2)
5. If the patient is unable to follow step 1 (oral diet) or step 2 (oral nutritional supplement) then the team should consider if an NGT is indicated.

Meal Plan A: Nutritional Rehabilitation

MEAL	STEP 1 Meals and Snacks <i>If not 100% consumed in 30 minutes (Meals) or 15 minutes (Snacks) then proceed to STEP 2</i>	STEP 2 Oral Supplement Drink <i>If not 100% consumed within 15 minutes then proceed to STEP 3</i> <i>Ensure correct product – no swaps</i>	STEP 3 NGT Bolus <i>Remainder of oral supplement drink to be provided via NGT bolus if inserted</i>
Breakfast 08:00-08:30	1 packet of cereal (30g) or porridge AND 1 bottle (225ml) full cream milk AND 2 cartons of fruit juice (220ml)	200ml Fresubin Energy	
Lunch 12:00-12:30	Hospital Main meal (standard serve) Must include a carbohydrate source (e.g. rice, pasta, potato) <u>and</u> protein <u>and</u> vegetables	200ml Fresubin Energy	
Dinner 17:00-17:30	Hospital Main meal (standard serve) Must include a carbohydrate source (e.g. rice, pasta, potato) <u>and</u> protein <u>and</u> vegetables AND Standard dessert <i>(must include ice-cream/yoghurt/custard)</i>	300ml Fresubin Energy <u>(1.5 bottles)</u>	
Snacks 10:00, 14:00, 19:00	200ml Fresubin Energy		
NOTE: Fortisip can be used as a substitute for Fresubin Energy if unavailable or preferred. 200ml = 1 bottle			

24.5 Appendix 5: Therapeutic Communication for patients with eating disorders

- Avoid critical or accusatory statements.
- **Focus on feelings and relationships**, not on weight and food.
- **Tell them you are concerned about their health but** respect their privacy. Eating disorders are often a cry for help, and the individual will appreciate knowing that you are concerned.
- **Do not comment on how they look.** The person is already too aware of their body. Even if you are trying to compliment them, comments about weight or appearance only reinforce their obsession with body image and weight.
- **Make sure you do not convey any fat prejudice** or reinforce their desire to be thin. If they say they feel fat or want to lose weight, don't say "You're not fat." Instead, validate that you understand this is a real concern for them and ask what would be supportive to distance them from those thoughts/feelings (e.g. activity, journaling).
- **Avoid power struggles about eating.** Do not demand that they eat. Do not criticize their eating habits. People with eating disorders are trying to be in control. They don't feel in control of their life. Trying to trick or force them to eat can make things worse.
- **Avoid placing shame, blame, or guilt** on the person regarding their actions or attitudes. Do not use accusatory "you" statements like, "You just need to eat." Or, "You are acting irresponsibly." Instead, use "I" statements. For example: "I'm concerned about you because you refuse to eat breakfast or lunch." Or, "It makes me worried to hear you vomiting."
- **Avoid giving simple solutions.** For example, "If you'd just stop, then everything would be fine!"

24.5.1 Meal support therapy

There are several points to keep in mind when planning Meal Support Therapy:

1. Coach with empathy and provide positive feedback:

- Validate the patient's struggle by acknowledging how emotionally and physically challenging it is.
- If the patient is not able to complete the snack or meal, find out why and discuss ways to make completion possible the next time, setting achievable goals.

2. Address the disordered eating behaviours at the time:

- Encourage once and leave it at that.
- Avoid using a confrontational tone.
- Be nurturing and supportive.
- Deal with what you observe directly (not with hearsay).

3. Distract during eating times:

- Be prepared and creative.
- Vary the distractions.
- Use background music to create a less tense atmosphere.

4. Be consistent:

- Be aware of meal plan and expectations.
- Do not bargain or negotiate with the meal plan.

- Be clear about what is expected.
- Follow the meal plan.

5. Use conversation that is non-emotional in tone during eating times:

- Refer to news items, word puzzles, mind games, horoscopes.
- Avoid conversations about food, diet and calories.
- Avoid talking about tube feeding and the need for nutritional supplements.
- Avoid personal issues.

When patients are admitted it is common and expected that they will struggle to eat meals/snacks.

Staff should acknowledge the struggle while at the same time setting limits with a firm, consistent approach. Patients have found certain phrases to be particularly helpful in allowing them to eat:

- “Your body really needs the fuel.”
- “This is your medicine.”
- “Take a few minutes to collect yourself, then start again. Try some relaxation and deep breathing.”
- “You must be angry and scared, but you deserve to eat. You deserve to get better.”
- “I see you are struggling. Right now, it is important that you get through the meal. Let’s take some time afterwards to talk about it.”

Meal Support Therapy is anxiety-provoking, but it gets easier over time. Staff who are positive role models and who consistently expect 100% completion of meals will see positive results.

24.6 Appendix 6: A Guide to applying the MHA 2014 for assessment and treatment of patients with an eating disorder

24.6.1 Using the Mental Health Act (MHA) 2014

Eating Disorders are mental illnesses that can be life-threatening and associated with impaired capacity due to the mental illness itself as well as the physical effects of starvation on the brain. The MHA can be used where the person's impaired capacity is putting them at risk and there is no less restrictive way of ensuring they receive treatment. Below is a list of the criteria for involuntary treatment under s25 of the MHA and notes to assist in deciding whether the criteria may apply to a patient with an eating disorder. To justify involuntary treatment, all criteria (a) to (d) must apply:

a) The person has a mental illness

Note: Anorexia Nervosa, Bulimia Nervosa, Other Specified Feeding or Eating Disorder (previously Eating Disorder Not Otherwise Specified), Avoidant Restrictive Food Intake Disorder (ARFID) and Un-specified Feeding or Eating Disorder are all listed as mental illnesses in DSM-V.

b) The person does not have capacity to consent to be treated for the illness

Note: In addition to the capacity-impairing symptoms of eating disorders such as intense fear of eating and/or gaining weight, starvation of the brain can cause profound cognitive changes such as rigid, inflexible thinking, obsessive preoccupation and fear around food and weight, ritualised behaviours around food and exercise, depression, and extreme decision-making difficulty. These capacity-impairing effects of starvation can be often reversed with nutrition. For more information, see *The Biology of Human Starvation* by Keys, Henschel & Brožek or, for a summary of the study go to [The Great Starvation Experiment, 1944-1945](#) or [The Effects of Starvation - Inside Out Institute](#).

c) Because of the person's illness, the absence of involuntary treatment is likely to result in imminent serious harm or serious mental or physical deterioration

Note: Anorexia Nervosa has the highest mortality rate (up to 20%) of any mental illness (See [About Eating Disorders - InsideOut Institute](#), 2023). Deaths can be due to reversible complications of malnutrition, eating disorder behaviours, and patient suicide. The Minnesota semi-starvation study demonstrated that starvation causes predictable mental, behavioural and physical symptoms that only reverse with nutritional rehabilitation (Refer to [The Great Starvation Experiment, 1944-1945](#).)

d) There is no less restrictive way for the person to receive treatment and care for their mental illness

24.6.2 Use of restrictive interventions

In determining whether a person is to be treated involuntarily under the MHA, clinicians must use the principles of trauma-informed care and consider least restrictive principles in providing treatment and care. An authorised doctor may make a Treatment order if treatment is considered necessary and no less restrictive options are appropriate to meet the person's needs. If involuntary treatment or care under the MHA is considered necessary in a medical ward, it is advisable to contact the MHCLS to seek assistance regarding the use of the MHA and discuss with the person and/or family/carers, if clinically appropriate.

24.6.3 Use of seclusion and restraint

Seclusion and physical restraint are to be used only as a last resort, where less restrictive interventions have been unsuccessful or are not feasible and according to the provisions of the MHA and the Chief Psychiatrist's Standards of Care, unless authorised under another law. For further information refer to the [Chief Psychiatrist of Western Australia website](#)

24.6.4 Guardianship and Administration Act 1990 (GAA)

If the person with an eating disorder has impaired capacity to provide or withhold consent to treatment, treatment without consent may also be possible under the GAA, if the criteria within the legislation are met.